

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 21

Age: 68 years old

Sex: Male

MRN: 000-TEST-21

Admission Date: [Admission Date]

Discharge Date: [Discharge Date]

Length of Stay: 4 days

****PRIMARY DIAGNOSIS****

Type 2 Diabetes Mellitus with hyperglycemic episode (admission glucose 487 mg/dL)

****SECONDARY DIAGNOSES****

- Hypertension (controlled)
- Chronic Kidney Disease Stage 3 (eGFR 42 mL/min/1.73m²)
- Hyperlipidemia
- Obesity (BMI 31.2)

****HOSPITAL COURSE****

Patient 21 presented via ED with polyuria, polydipsia, and generalized fatigue x 3 days. Initial labs revealed glucose 487 mg/dL, HbA1c 9.8%, serum creatinine 1.7 mg/dL. Urinalysis negative for ketones; no DKA. Patient was admitted for management of hyperglycemic episode and optimization of glycemic control.

During stay:

- IV fluids (0.9% NS) initiated for mild dehydration; total 2L over 24h
- Insulin drip started (0.05 units/kg/hr); glucose trended to 180–220 mg/dL by hospital day 2
- Transition to basal-bolus regimen completed by day 3
- Renal function monitored q.d.; creatinine stable at 1.68 mg/dL
- Nephrology consult recommended CKD management; albuminuria noted (urine albumin-to-creatinine ratio 65 mg/g)
- Diabetes education provided; foot exam performed—no ulceration or neuropathy noted
- Lipid panel: Total chol 218, LDL 142, HDL 38, TG 215 mg/dL

Patient tolerated PO diet well by discharge. Ambulating without assistive device. Discharged in stable condition with close outpatient follow-up arranged.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
------------	------	-----------	----------

---	---	---	---
-----	-----	-----	-----

Insulin Glargine	20 units	Once daily at bedtime	Ongoing
------------------	----------	-----------------------	---------

Insulin Lispro	6 units	Before meals (TID)	Ongoing
----------------	---------	--------------------	---------

Metformin	500 mg	BID with meals	Ongoing
-----------	--------	----------------	---------

Lisinopril	20 mg	q.d.	Ongoing
------------	-------	------	---------

Amlodipine	5 mg	q.d.	Ongoing
------------	------	------	---------

Atorvastatin	40 mg	q.d. at bedtime	Ongoing
--------------	-------	-----------------	---------

Aspirin	81 mg	q.d.	Ongoing
---------	-------	------	---------

****GLUCOSE MONITORING & DIETARY INSTRUCTIONS****

Fasting and pre-meal fingerstick glucose checks four times daily (AC and HS). Record all readings in provided log. Bring log to all appointments.

Diet: Consistent carbohydrate intake (45–60g per meal), low-sodium (<2g/day) for HTN, low-phosphorus diet given CKD stage 3. Avoid simple sugars and refined carbohydrates. Dietitian referral placed; pt to call for appointment.

****ACTIVITY & FOOT CARE****

- No strenuous activity × 1 week; resume normal activity as tolerated
- Foot inspection daily (morning & evening) for redness, blisters, cracks
- Wash feet with lukewarm water BID; apply moisturizing lotion to dry skin (avoid between toes)
- Wear